TASK 5 – INDIVIDUAL CASE SAFETY REPORT (ICSR)

VirtualWorks Lab | Pharmacovigilance Internship | Task ID: task5

1. Objective

To perform an integrated pharmacovigilance assessment of a suspected adverse drug reaction (ADR), including ADR identification, ICSR documentation, causality assessment, and seriousness/severity/expectedness classification.

2. Case summary

Field	Available case information
Patient	45-year-old female
Suspected medicine	Amoxicillin
Reported reaction	Skin rash and itching
Time to onset	3 days after starting treatment
Action / dechallenge	Medicine stopped; recovery reported afterwards
Outcome	Recovered

The case facts above are drawn from the earlier task material showing a 45-year-old female with amoxicillin-associated skin rash and recovery after stopping the medicine. They are used here as the available working scenario; no additional patient facts have been assumed.

3. ADR identification

Suspected ADR: Skin rash and itching following amoxicillin use.

Temporal relationship: The reaction reportedly began three days after treatment initiation. This timing is compatible with a possible drug-related reaction.

Dechallenge: Recovery was reported after amoxicillin was stopped. This supports an association, although it does not prove causality by itself.

Alternative causes: Not assessed because information about the underlying infection, other medicines, allergy history and other possible causes is unavailable.

4. ICSR documentation – structured case fields

ICSR field	Entry
Patient initials / identifier	Not provided
Age / sex	45 years / Female
Weight / date of birth	Not provided
Relevant medical history	Not provided
Suspected medicinal product	Amoxicillin
Dose / frequency	Not provided
Route / indication	Not provided
Treatment start / stop dates	Calendar dates not provided; reaction occurred 3 days after treatment initiation; medicine subsequently stopped
Adverse reaction	Skin rash and itching
Reaction onset date	Exact calendar date not provided; day 3 after treatment initiation
Reaction end / recovery date	Exact date not provided; recovery reported after withdrawal
Seriousness	No serious criterion described; provisionally non-serious
Outcome	Recovered
Concomitant medicines	Not provided

ICSR field	Entry
Treatment for reaction	Not provided
Reporter identity / contact	Not provided; complete only with authorized reporter details

Minimum ICSR validity elements should be checked before treating a report as a valid case: an identifiable patient, an identifiable reporter, a suspected medicinal product, and a suspected adverse reaction. The available training scenario does not provide enough information to verify all four elements. Reporter details must not be invented.

5. Causality assessment

Naranjo Algorithm: A reliable total score cannot be calculated from the limited case description because several questionnaire items are unknown, including alternative causes, rechallenge, prior similar reactions, dose-response information and objective confirmation. Do not assign unsupported points.

WHO–UMC assessment: Possible (provisional). The timing is compatible and recovery after withdrawal supports a relationship. However, alternative causes have not been adequately evaluated, so the available information does not establish a Probable/Likely or Certain relationship.

6. Seriousness, severity and expectedness

Seriousness: Provisionally non-serious based on the information supplied. No death, life-threatening event, hospitalization, persistent/significant disability, congenital anomaly or other medically important event is described. If further information establishes any such criterion, reassess.

Severity: Cannot be graded as mild, moderate or severe from the brief description alone. The extent, intensity, treatment required and impact on daily activities are not supplied.

Expectedness: Skin rash is a recognized reaction associated with amoxicillin, so it is likely expected in general. Formal expectedness must be checked against the applicable product-specific reference safety information and its version.

7. Medical case narrative

A 45-year-old female reportedly developed skin rash and itching three days after starting treatment with amoxicillin. The suspected medicine was subsequently discontinued, and the patient was reported to have recovered. The temporal relationship and reported improvement following withdrawal support a possible association between amoxicillin and the reaction. However, the available case information does not include the dose, route, indication, exact treatment and event dates, concomitant medicines, relevant medical history, treatment of the reaction, or details needed to assess alternative causes. The seriousness is provisionally considered non-serious because no serious outcome is described; severity cannot be graded from the information available. The expectedness assessment should be confirmed against the applicable product-specific reference safety information.

8. ICSR processing and quality review

1. Receive and record the report and the organization's initial receipt date (Day 0).
2. Verify the four minimum ICSR validity elements.
3. Search for possible duplicate cases.
4. Enter patient, suspect medicine, reaction, dates, outcome and reporter information.
5. Code the reaction using the current MedDRA terminology and applicable coding conventions.
6. Assess seriousness, expectedness and causality using the available evidence.
7. Perform data-entry quality control and medical review.
8. Determine the applicable reporting destination and regulatory timeline before submission.

9. Conclusion

The available information describes a suspected amoxicillin-associated skin rash and itching in a 45-year-old female, with onset three days after treatment initiation and recovery after discontinuation. The case supports a provisional WHO–UMC Possible assessment. Seriousness is provisionally non-serious; severity cannot be determined from the brief description; expectedness is likely but requires product-specific confirmation. Additional information is needed to complete and validate the ICSR. No regulatory submission should be made on the basis of this incomplete training scenario.

10. References

1. International Council for Harmonisation (ICH). E2A: Clinical Safety Data Management – Definitions and Standards for Expedited Reporting. ICH E2A guideline

2. World Health Organization – Uppsala Monitoring Centre. The use of the WHO–UMC system for standardised case causality assessment. WHO–UMC causality assessment

3. Indian Pharmacopoeia Commission. Pharmacovigilance Programme of India (PvPI). PvPI resources and toolkit

4. Medical Dictionary for Regulatory Activities (MedDRA). MedDRA